

Sinusitis – Summary

Key Symptoms: Pain/pressure in face, nasal congestion/discharge, fever, altered taste/smell, earache, headache, irritability, etc. (see slide 6)

Most common causing-pathogens

- **Acute sinusitis:** Viral > bacterial: *S. pneumoniae*, *H. influenzae*, *M. catarrhalis*
- **Chronic sinusitis:** *S. aureus*, enteric gram-negative bacteria, *Pseudomonas* spp., fungal

Non-Pharmacological Treatment: Nasal irrigation, rest & hydration, regular handwashing, avoid allergen exposure, sleep with head elevated, apply warm facial compress

When to Treat:

- Symptoms <7 days → symptomatic treatment
- Symptoms >7 days → PODS ≥ 2 → following points:
 - **Mild/mod:** Intranasal corticosteroids
 - **Severe:** Oral antibiotics
 - ★ **P:** Pressure, facial **Pain**
 - ★ **O:** nasal **O**bstruction
 - ★ **D:** Discoloured/purulent nasal or post-nasal **D**ischarge
 - ★ **S:** hyposmia/anosmia (**S**mell)

Pharmacological Treatment

- Symptomatic relief: decongestants, antihistamines, nasal corticosteroids, analgesics and antipyretics
- **Abx**
 - **Uncomplicated:** 5-7 day Tx for adults, 10-14 days in children
 - **Adult:** Amoxicillin
 - **2nd line:** amoxi-clav, doxycycline, respiratory fluoroquinolone (FQ) e.g. levofloxacin/moxifloxacin, SMX-TMP, macrolides, ceftriaxone
 - **Children:** Amoxicillin (45-90 mg/kg/day)
 - **2nd line:** Amoxi-clav, SMX-TMP, macrolides, cefixime + clindamycin, doxycycline (>8 y)
 - **Resistance/Tx failure:** Amoxi-clav
 - **2nd line:** Respiratory FQ (levofloxacin/moxifloxacin)

Monitoring Parameters: Improvement in 2-3 days, resolution within 1 week

Additional Key Points

- Generally, would avoid antihistamines as anticholinergic side effects may dry the mucosa and prevent secretion and drainage, but useful in allergic sinusitis
- Limit use of intranasal decongestants to 3 days due to risk of rebound congestion (and avoid in children <6 years)
- Suspect treatment failure if symptoms worsen after 72 hours of treatment initiation, reassessment required